

PAC2508-0006
Leaflet_KLF0026S1_Camazol Tablet 5mg
Size: 148(W) x 210(H)mm
Colors: Black
Current Revision: 004
Updated Revision: 005

Important information. Please read carefully.

CamazolTM
Tablets

COMPOSITION
Each tablet contains Carbimazole 5mg

PHARMACODYNAMICS
Carbimazole is an anti-thyroid agent which acts by interfering with the incorporation of iodine into thyroglobulin in the thyroid gland.

PHARMACOKINETICS
Carbimazole is absorbed from the gastrointestinal tract and is widely distributed throughout the body.
It crosses the placenta readily and is present in the milk during lactation.
It is metabolised to methimazole, which is an active metabolite, and is excreted in the urine.
Excretion is almost complete in 24 hours.
Methimazole is appreciably bound to plasma proteins and is reported to have an extended plasma half-life in hypothyroid subjects, but a reduced half-life in hyperthyroid subjects.

INDICATIONS
It is used for the treatment of thyrotoxicosis, Grave's disease and in the preparation of subtotal thyroidectomy.

DOSAGE AND ADMINISTRATION
To be administered orally.
Thyrotoxicosis : Initially, a higher dose should be given to quickly render the patient euthyroid, and then gradually reduces to a minimum effective dose for maintenance.
Suggested dosage for adults :
Initial dose : 10 to 60mg daily in divided doses.
Doses may be given at eight hour intervals.
Maintenance : 5 to 20mg daily.
Suggested dosage for children:
1 to 6 years : 7.5mg daily in divided doses as initial dose.
above 6 years :15mg daily in divided doses as initial dose.
Preparation of thyrotoxic patient for surgery : **Camazol** should be given in sufficient dosage to render the patient euthyroid, and continued up to the time of operation. Iodine should be prescribed together during the last two weeks.
Dosage should be adjusted by the physician according to the response of the patient.

CONTRAINDICATIONS
Contraindicated in patients with blood dyscrasias and those who have shown hypersensitivity towards carbimazole. Do not administer in patients with tracheal obstruction as high dosage may lead to thyroid enlargement and aggravate the condition. Patients with a history of acute pancreatitis after administration of carbimazole or active metabolite, methimazole (thiamazole).

PRECAUTIONS
Although there is a risk to the foetus in pregnancy especially in high doses, it may be given with the smallest effective dose and given close medical supervision. Where possible, discontinue treatment 3 to 4 weeks before delivery.
Carbimazole may be transferred to breast milk and if possible avoid breast feeding. However, if neonatal development is closely monitored, breast feeding may be carried out. It is dangerous to exceed the stated dose.

WARNING
Carbimazole may cause white cell disorders such as neutropenia and agranulocytosis, which may be fatal if treatment with carbimazole is not stopped promptly. These reactions usually occur during the first 3 months of therapy, and in most cases, are reversible on stopping treatment. Since agranulocytosis can be effective in the early detection of these reactions.

During treatment with **Camazol** :
1. The patient should be asked to immediately report signs and symptoms suggestive of infection, e.g. sore throat, fever, mouth ulcer.
2. A white cell count should be performed if there is any clinical evidence of infection.
3. The drug should be stopped promptly if there is clinical or laboratory evidence of neutropenia.
Following the onset of any signs and symptoms of hepatic disorder (pain in the upper abdomen, anorexia, general pruritus) in patients, the drug should be stopped and liver function tests performed immediately. Early withdrawal of the drug will increase the chance of complete recovery.
Carbimazole tablets should be used with caution in patients with mild- moderate hepatic insufficiency. If abnormal liver function is discovered, the treatment should be stopped. The half-life may be prolonged due to the liver disorder.

Pancreatitis
There have been post-marketing reports of acute pancreatitis in patients receiving carbimazole or its active metabolite, methimazole (thiamazole).
In case of acute pancreatitis, carbimazole or methimazole (thiamazole) should be discontinued immediately. Carbimazole or methimazole (thiamazole) must not be given to patients with a history of acute pancreatitis after administration of carbimazole or its active metabolite methimazole (thiamazole). Re-exposure may result in recurrence of acute pancreatitis, with decreased time to onset.
Women of childbearing potential and pregnancy
Women of childbearing potential have to use effective contraceptive measures during treatment.
The use of carbimazole or methimazole (thiamazole) in pregnant women must be based on the individual benefit/risk assessment. If carbimazole or methimazole (thiamazole) is used during pregnancy, the lowest effective dose without additional administration of thyroid hormones should be administered. Close maternal, foetal, and neonatal monitoring is warranted.

Pregnancy and Lactation
Women of childbearing potential
Women of childbearing potential have to use effective contraceptive measures during treatment (see *Section Warnings and Precautions*).
Pregnancy
Carbimazole or methimazole (thiamazole) crosses the placenta but, provided the mother's dose is within the standard range and her thyroid status is monitored; there is no evidence of neonatal thyroid abnormalities.
Studies have shown that the incidence of congenital malformations is greater in the children of mothers whose hyperthyroidism has remained untreated than in those who have been treated with carbimazole or methimazole (thiamazole).
However, cases of congenital malformations have been observed following the use of carbimazole or its active metabolite, methimazole (thiamazole) during pregnancy.
A causal relationship of these malformations, especially choanal atresia and aplasia cutis congenita (congenital scalp defects), to transplacental exposure to carbimazole and methimazole (thiamazole) cannot be excluded.
Therefore the use of carbimazole or methimazole (thiamazole) in non-pregnant women of childbearing potential should be based on individual risk/benefit assessment (see section Warnings and Precautions).
Cases of renal, skull, cardiovascular congenital defects, exomphalos, gastrointestinal malformation, umbilical malformation and duodenal atresia have also been reported. Therefore, carbimazole or methimazole (thiamazole) should be used in pregnancy only when propylthiouracil is not suitable.



If carbimazole or methimazole (thiamazole) is used in pregnancy, the dose must be regulated by the patient's clinical condition. The lowest dose possible should be used, and this can often be discontinued three or four weeks before term, in order to reduce the risk of neonatal complications.
The blocking-replacement regimen should not be used during pregnancy since very little thyroxine crosses the placenta in the last trimester. Hyperthyroidism in pregnant women should be adequately treated to prevent serious maternal and foetal complications.
Carbimazole or methimazole (thiamazole) is able to cross the human placenta.
Based on human experience from epidemiological studies and spontaneous reporting, carbimazole or methimazole (thiamazole) is suspected to cause congenital malformations when administered during pregnancy, particularly in the first trimester of pregnancy and at high doses.
Reported malformations include aplasia cutis congenita, craniofacial malformations (choanal atresia; facial dysmorphism), exomphalos, oesophageal atresia, omphalo-mesenteric duct anomaly, and ventricular septal defect.
Carbimazole or methimazole (thiamazole) must only be administered during pregnancy after a strict individual benefit/risk assessment and only at the lowest effective dose without additional administration of thyroid hormones. If carbimazole or methimazole (thiamazole) is used during pregnancy, close maternal, foetal and neonatal monitoring is recommended (see *section Warnings and Precautions*).

DRUG INTERACTIONS
Little is known about interactions.
Interaction studies have not been performed in paediatric patients.
Particular care is required in case of concurrent administration of medication capable of inducing agranulocytosis.
Since carbimazole is a vitamin K antagonist, the effect of anticoagulants could be intensified. Additional monitoring of PT/INR should be considered, especially before surgical procedures.
The serum levels of theophylline can increase and toxicity may develop if hyperthyroidic patients are treated with antithyroid medications without reducing the theophylline dosage.
Co-administration of prednisolone and carbimazole may result in increased clearance of prednisolone.
Carbimazole may inhibit the metabolism of erythromycin, leading to reduced clearance of erythromycin.
Serum digitalis levels may be increased when hyperthyroid patients on a stable digitalis glycoside regimen become euthyroid; a reduced dosage of digitalis glycosides may be needed.
Hyperthyroidism may cause an increased clearance of beta-adrenergic blockers with a high extraction ratio.
A dose reduction of beta blockers may be needed when a hyperthyroid patient becomes euthyroid.

INCOMPATIBILITIES
None known.

SIDE EFFECTS
Common side effects which usually occur in the first 8 weeks of treatment may include skin rashes, nausea, headache and gastrointestinal upsets. Other adverse effects sometimes observed include fever, arthralgia, myopathy, myalgia, a lupus-like syndrome, vasculitis and nephritis, taste disturbances, abnormal hair loss, pruritus and urticaria. Hepatic disorders, including abnormal liver function tests, hepatitis, cholestatic hepatitis, cholestatic jaundice and

most commonly jaundice, have been reported; in these cases carbimazole tablets should be withdrawn. Rare cases of pancytopenia/ aplastic anaemia and isolated thrombocytopenia have also been reported. Very rare cases of haemolytic anaemia have been reported. Patients must be advised to report incidence of sore throat immediately as agranulocytosis may be encountered. Over-treatment may lead to hypothyroidism and should be avoided in pregnancy as this may cause foetal goitre.
SOC Gastrointestinal disorders
Frequency "Not Known": Acute pancreatitis.

OVERDOSAGE
In cases of overdosage and poisoning, hospital admission is strongly advised. Symptoms include skin rashes and leucopenia. Treatment is symptomatic and supportive.

SHELF-LIFE
The expiry date is indicated on the packaging.

PRESENTATION
White, round, flat bevelled edge tablet with one breakline on one side, 6.3 mm in diameter. Available in packs of 100's and 1000's.

STORAGE
Store below 25°C. Protect from light.

KEEP OUT OF REACH OF CHILDREN
JAUHI DARI KANAK-KANAK

For further information, please consult your pharmacist or physician.

Revision Date: 19-Aug-2025

Manufacturer and Product Registration Holder:
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148mm

210mm